

Stock Pitch:

EPS: 9.14; EPS-NY: 22.66;

Lst. Q Rev: \$13.5B; Lst. Q Op Income: \$5.1B;

Last Y Revenue: \$45B; Last Y. Op Income: \$17B

EV/EBITDA: 47.56; EV/EBIT: 66.6

PE: 75.44; F PE: 29.99;

DSO: ~65; DPO: ~85

Market Cap: \$801B

EV: \$777B

Stock has outperformed the S&P 500 by 21% over the past year, 1241% over the past decade

CEO: David Ricks, CFO: Lucas Montarce

1. Introduction:

- a. I will pitch a long on Eli Lilly, which trades at a 52-week high/low of 711-972 on the NYSE under the symbol LLY.

2. Background:

- a. For some background, Eli Lilly is a drug manufacturer founded in 1876 in Indianapolis, IN. The company has three main focus areas: cardiometabolic health, oncology, and immunology.

3. Reasons:

- a. In the past few quarters, Eli Lilly issued lower guidance and reported a miss due to inventory issues, which caused the stock to undergo a 25% correction. I think the street has overestimated the adverse effects of Eli Lilly's inventory problems and that there is a significant upside remaining in the stock because:

- i. **Customers are choosing Lilly over other GLP-1 drugmakers in this fast-growing market** (CAGR 21.3% (Polaris market research) between 2025 and 2034)

1. Eli Lilly brought 47% more weight loss, according to Yahoo, compared to Wegovy, the leading competitor
2. CNBC Article: 'Mounjaro and Zepbound are expected to bring in \$50 billion alone by 2028 – almost twice the company's full-year revenue in 2022.'

- a. Zepbound: 11% of Revenues (grew ~40% YoY)
- b. Mounjaro: 27.21% of Revenues (grew ~40% YoY)

- ii. **Eli Lilly is protected against Patent Expiry with a robust future-proof drug pipeline** that brings stand-out drugs like an Oral GLP-1 and IBD Drugs

1. Zepbound/Mounjaro Patent Expiration: May 2026
2. Omvoh (IBD) Patent Expiration: March 2034

3. New Drugs:

- a. Orforglipron– Oral weight loss pill more effective than Wegovy, less effective than is expected to launch in 2026
- b. Retatrutide (expected approval 2025): Injectable weight loss drug that offers a higher weight loss (22-23% vs 22%) compared to ZepBound
- c. Morf-057 (Q2 2025 Phase 2 expected)

4. Growth Rate of R&D Expected: +18% YoY into the future

iii. Eli Lilly is addressing investor concerns regarding inventory by making significant investments in manufacturing facilities and partnering with CDMOs

- 1. Eli Lilly has made \$23B in capex investments, notably \$4.5M in Indiana and \$1.8M in Ireland, that will help it generate 60% more revenue this upcoming year from incretins/GLP-1
- 2. Eli Lilly partnered with CDMO BSP Pharmaceuticals and National Resilience to fill/finish its GLP-1 drugs, primarily offsetting Novo Nordisk's acquisition of Catalent, granting NVO 50 extra facilities.

4. Risks:

- a. The biggest risk with Eli Lilly was that of compounded drugs
 - i. **Mitigation:** The compounded drug makers were told to stop production and sales of compounded GLP-1s by March 2025. The new administration has shown no indication to reverse this decision, and they are looking to cover GLP-1 drugs with Medicare and Medicaid at a cost of \$35B, but with potential savings of \$175B according to a USC study.

5. Conclusion:

- a. Based on these factors, Lilly is a strong buy given its market positioning, drug pipeline, and competent management team.

Expected Follow Ups and Clarifications:

1. What drives Eli Lilly's Revenue Growth?

- a. Eli Lilly's revenue grew 32% over the past year, driven primarily by sales of Monjaro and Zepbound. Management expects this to continue over the coming year, with forward revenue guidance of \$58B of revenue, assuming a base case of 32% growth

2. Who are Eli Lilly's Geographic Breakdown and Customer Concentration?

- a. Geographic Breakdown:
 - i. USA: 64%
 - ii. Europe: 18%; Grew 61% YoY, hedging out through FX securities
 - iii. Japan: 4.9%
 - iv. China: 4.5%

v. Others: 8.6%

3. *Who are Eli Lilly's main competitors? How does the company differentiate itself from the competition?*

- a. Eli Lilly's primary competition includes companies such as Novo Nordisk, Amgen, and Abbvie.

4. *There is talk surrounding the decline in GLP-1s and in general, that this industry has and will experience a slowdown in growth?*

- a. Yes, an analyst named Steve Scala at TD Cowen argued this point. Truthfully, we are in the infancy of seeing what GLP-1 drugs can do. They can treat so much more than just Obesity based on what we are seeing with Zepbound, Liraglutide, and other drugs like KP-405.

5. *Will these other drugs gain FDA approval with the new administration?*

- a. In their current state, I think not, and here is why.
- b. One of the biggest risks with GLP-1s is the digestive system's difficulty tolerating their side effects. Pfizer's weight loss drug (Danuglipron) was discontinued because more than 50% of patients quit the study because of how badly its patients suffered from nausea and cramps.
- c. With a stricter administration under Trump and RFK Jr., it is **less likely** for a drug to get past phase III if it cannot top Zepbound/Monjaro's relatively low discontinuation rate.

6. *Is Eli Lilly a 'one trick pony' purely for GLP-1 drugs? What are some other drug areas that the company focuses on?*

a. *Answer:*

- i. Eli Lilly is so much more than a GLP-1-focused drug company. Their oncology and immunology businesses grew 20% over the past year, with blockbuster drugs like Verzenio and MORF-057. ***How has Eli Lilly combated drug failures in the past?***

7. *Can you breakdown the CAGR of 33%?*

- a. The CAGR is primarily driven by obesity versus diabetes because the market is larger.